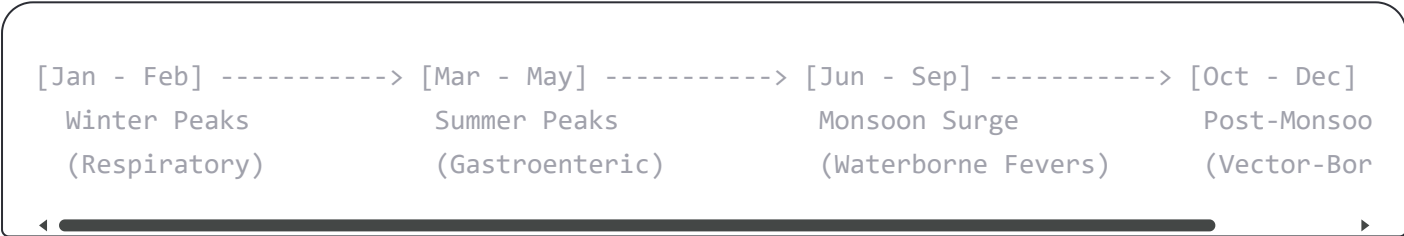


Recent epidemiological data and reports from the Integrated Disease Surveillance Programme (IDSP), Brihanmumbai Municipal Corporation (BMC), and the Maharashtra Public Health Department's Arogya Patrika highlight a distinct seasonal pattern in disease transmission .  ResearchGate +2

In recent years (**2024–2026**), climate changes—particularly **extended monsoons** and **early onset of rains**—have amplified the volume of waterborne and vector-borne illnesses, shifting historical timelines.  LinkedIn +1

The Three-Phase Seasonal Disease Pattern

Analysis of monthly health surveillance data reveals that India (and Maharashtra specifically) experiences three distinct disease waves annually:



- 1. **The Monsoon & Post-Monsoon Surge (June to November):** This is historically the **highest public health burden period**. It begins with waterborne outbreaks and transitions into severe vector-borne (mosquito) epidemics.
- 2. **The Winter Peak (December to February):** Dominated by viral respiratory infections.
- 3. **The Summer Peak (March to May):** Characterized by food poisoning and classic gastrointestinal spikes due to heat and water scarcity.  arXiv +4

Detailed Monthly Disease Matrix

The following table synthesizes the typical monthly trajectories based on recent data from Maharashtra and all-India sentinel surveillance:  impactfactor.org +2

Months	Predominant Diseases	Peak Drivers / Climate Correlates	Recent Surveillance Observations (2024–2026)
January – February	Influenza (H1N1), Pneumonia, RSV	Lower temperatures, high humidity, viral stability	Late winter bimodal influenza peak recorded across states.

March – May	Acute Diarrheal Disease (ADD), Food Poisoning, Typhoid	Water scarcity leading to contaminated supply pipelines	Early summer spikes in ADD outbreaks across central India.
June – July	Cholera, Gastroenteritis, Leptospirosis, Malaria	Initial heavy rain, water logging, wading in flooded streets	Maharashtra recorded a 3x surge in waterborne outbreaks by July/August compared to early-year baselines. Malaria increased significantly due to early monsoons.
August – September	Dengue, Chikungunya, Typhoid	Stagnant rainwater, optimal mosquito vector breeding humidity	Dengue and Malaria consistently peak in September. Typhoid claims rose by 37% due to prolonged monsoon logging.
October – November	Dengue (Late peak), Scrub Typhus, Malaria	Post-monsoon stagnation	Extended vector activity stretching well into November.
December	Respiratory Infections, Influenza	Onset of winter dip in temperatures	Transition into winter viral bimodal loop.

Key Macro Trends Identified in Recent Reports

1. The Vector-Borne Escalation (Dengue & Malaria)

Data from Mumbai's 2026 monsoon health report revealed a significant year-over-year escalation compared to 2025:  Facebook

- **Malaria cases** increased by **18.2%** (jumping from 3,115 to 3,681 cases mid-year).
- **Dengue cases** saw a **27.8% surge**. The PharmEasy Fever Watch and laboratory data show that Aedes mosquito breeding heavily concentrates around urban micro-habitats (like decorative plants and uncovered tanks), pushing the dengue positivity timeline tightly into the August–October window.  PharmEasy +3

2. Shifting Typhoid Dynamics

Typically, viral fevers dominate the monsoon. However, recent insurance and hospital data tracked a **37% increase in Typhoid cases**. This is directly tied to the *prolonged* nature of recent monsoon cycles, which continuously compromise drinking water infrastructure over several months.  NCDC | MOH... +1

3. Bimodal Influenza Patterns

Recent clinical updates from tertiary care facilities highlight that Influenza (specifically Influenza A) operates on a distinct **biannual (bimodal) cycle** in India. Instead of a single winter peak, it creates a massive wave in **August–September** (late monsoon) and a secondary wave in **March** (late winter), completely skipping the dry summer months of April to June.  Cureus

If you are analyzing this for a specific public health or research objective, please let me know:

- Do you want to focus on a **specific city or region** (e.g., Mumbai, Pune, rural Maharashtra)?
- Are you looking for statistics on a **particular disease class** (e.g., vector-borne vs. waterborne)?
- Do you need **historical comparisons** to pre-pandemic baselines?